جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ماجده حسين محمد طلبه
المحافظة : القاهره **الحي / المدينة :** حلوان **العنوان :** المعصره
رقم البطاقة : 25411260103223 **رقم التليفون :** 01003157775 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 26-11-1954 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ماجده حسين محمد طلبه
National ID: 25411260103223

GUSTAVE ROUSSY INTERNATIONAL EGYPT

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

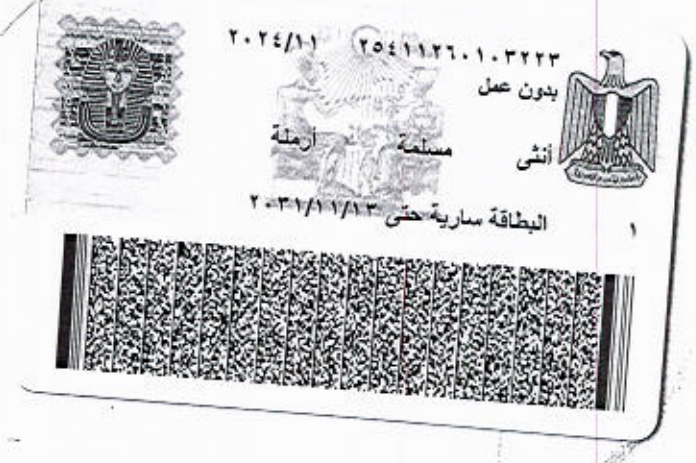
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Supportive and palliative Treatment**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
ماجده
حسين محمد طلبه
٣-١ ش ابو الوفا - المعصره البلد
المعصره - القاهره
٢٣ ٠٣٢ ٠١ ٢٦ ١١ ٢٥٤
KV8893104


٢٥٤١١٢٦٠١٠٣٢٢٣
٢٠٢٤/١٠١
بدون عمل
أنثى
مسلمة
أرملة
البطاقة سارية حتى ٢٠٣١/١١/١٣

Name: ماجده حسين محمد طلبه	
MR. : 6501	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is receiving adjuvant hormonal therapy with Femara and Zometa for six months, with Femara continued for an additional three months and Zometa administered today following laboratory evaluation.

The patient, a 71-year-old female, underwent upfront radiation therapy and modified radical mastectomy in April 2024. Pathology revealed PT2N3 disease with ER 8/8, PR 3/8, and HER2 negative status. She received adjuvant chemotherapy, a radiation port, aromatase inhibitor therapy, and Zometa for six months. Femara was continued from November 2024 for an additional three months.

تحريرا في ,

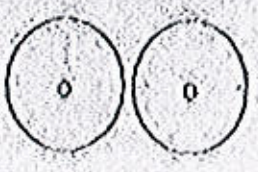
التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : **Ali Adel**

يعتمد ,

MDT form

A-Patient ID: [redacted]
Name: [redacted] Age: [redacted] Sex: Female
B-ID number: 35411210103733
3- Medical number: 03185237 OP/IP: OP Date: 3/5/2024
Patient Age: 63 Y



Family history	Physical eye	Pharm	Pan 01007851067	BMI	PS	REASON OF NOT	Non-local recurrence
marital status	Previous surg	IIRS Score	Weight	GROUP	Date	Date of NOT	Comment 11/05/2024
Co-morbidity	Systemic diseases	Menstrual status	Previous Pan Tx	Clinical STAGE	Weight		

Clinical data

Present complaint:
RT axillary recurrent at 9 at 10 o'clock + level enlarged LN

Investigations:
ct: pulmonary nodule likely mets

BX	ER	PR	Her 2 Neu	SISH	Ki67
ILC	8/8	3/8	1		

T	N	M
3	1	0

Sonomammography

PATIENT HISTORY
RT MRM:IDC , ILC G1I
focal DCIS
+ve lymphovascular invasion
+ve perineural invasion
free nipple
LN 16/20 with extracapsular extension
resection margins:focally nearby linked deep margin

MDT OUTCOME & PLAN

Treatment plan
for adj EC x4 , taxol x12
PORT
AI x 5 years + zometa /6 mns

SIGNATURE

MEDICAL ONCOLOGY	Dr. [redacted]	RADIOLOGY	Dr. [redacted] 8/5/24
SURGERY		PATHOLOGY	
Psychiatric		Radiation	

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	الأسم / مايادة حسن محمد طلبة	Request No.	13001528
Patient No.	25411260103223	Request Date	03/03/2024
Path No.	SR-303/24	Verification Date	18/03/2024
Sex	Female	Financial Category	برنامج دار السلام - نقابة الأطباء
Age	69 Y	Department	Out Patient
Referreing Doc.		Registered by	Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass.
Nature of Specimen: Slide Revision

Gross Examination:
- One paraffin block labeled by (732/1/24) was received.

Microscopic Examination:
- Histological type: Invasive lobular carcinoma.
- Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
- DCIS: No.
- Angioinvasion: No.
- Perineural Invasion: No.
- IPT:
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Weak nuclear reaction in 5% of tumor cells (score 3/8).
- Her2: Faint incomplete membrane reaction in >10% of tumor cells (score 1+).

Diagnosis:
Right breast mass, Slide revision:
Invasive lobular carcinoma, Grade II.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 3/8).
- Her2: (score 1+).

Comment: Although Her2 is Negative, Such cases (Score 1+) are predictive of response to Her2-Ab drug conjugates (ADCS) according to latest 2023 ASCO-CAP guidelines.

Verified by :
Dr. Sagda El-Kady

Room No. (4) - First Floor Building No. 237 - Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: [illegible]	Request No.	: 13009527
Patient No.	: 2[illegible]11260103223	Request Date	: 06/04/2024
Path No.	: 1255/24	Verification Date	: 21/04/2024
Sex	: Female	Financial Category	: [illegible]
Age	: 69 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, Up-Front

Nature Specimen: Right modified radical mastectomy specimen.

Gross Examination:
- Procedure: Right MRM measured 26x17x5 cm with overlying skin ellipse measured 22x9 cm.
- Tumor extent: An ill-defined greyish white firm mass, located outer quadrant.
- Size: 4.5 x4.5 x2 cm.
- Skin involvement: Nipple & areola were unremarkable grossly.
- Distance from margins: 0.1 cm away from inked deep margin.
- Lymph nodes number and size: 20 L.Ns up to 3x2.5x1 cm.
- Rest of breast tissue showing grayish white areas with beaded foci.
- There was portion of sutured attached fibrofatty tissue piece measured 9x8x1 cm, not overlying the mass site, looked un remarkable grossly.
- Also received fibrofatty tissue piece measured 4x2.5x1 cm, looked un remarkable grossly.

Microscopic Examination:
- Histological type: Mixed Invasive duct and lobular carcinoma.
- Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- Ductal carcinoma in situ: Focal DCIS (comedo, non comedo 10%).
- Lymphovascular emboli: Positive.
- Perineural invasion: Positive.
- Skin involvement: Free nipple.
- Lymph node involvement: Positive in (16/20) with positive extracapsular extension.
- Resection Margins: focally nearby inked deep margin.
- Microcalcifications: No.
- Rest of the breast showed fibrocystic changes.

Verified by

Patient's Name:	**Magda Hussein Mohammed Talba**
Date:	**2026-09-09**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Average in size (15cm at Rt MCL) homogenous parenchymal echopattern with smooth surface.*
- *No hepatic focal lesions or intra hepatic biliary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- Average distention and capacity with no stones or mud
- *. The wall is not thickened.*
- *CBD is not dilated.*

Spleen: -

- *Average size, homogenous.*

- **Kidneys: -**
  - *Both are average in size, shape , both show mild increase in echogenicity*
  - *No stones or dilated pelvicalyceal system*
  - *No gross lesion*

***Para-aortic and pancreatic area**: show no gross abnormalities.*

Moderate ***ascites***

Urinary Bladder: -

- *empty*

Opinion:

Bilateral grade I nephropathy for lab correlation

Moderate ascities

Dr/Nermeen ramadan.MD

Ultrasound
[[DOCTYPE:ULTRASOUND]]

Patient's Name:	**Magda Hussein Mohammed Talba**
Date:	**2026-08-04**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**



MULTISLICE CT EXAMINATIUON OF THE CHEST ,ABDOMEN AND PELVIS WITHOUT CONTRAST ADMINISTRATION REVEALED:-

- A right thyroid nodule is seen and measures about 20x19mm in dimensions.
- Evident right breast mastectomy with clear operative bed . No evidences of recurrent and /or residual neoplastic lesions seen.
- **Normal left breast.**
- **Multiple variable sized left axillary lymph nodes are seen with loss of their hilar fat contents in group of them and thickened cortex and the largest one measures about 24 mm.**
- Hyperinflated both lungs.
- Bilateral apical pleural capping is still seen more on the right side with fibrotic bands .ill-defined pulmonary nodules seen herein with the largest one measures about 4mm.
- Mild subpleural reticulations are seen in the anterior segment of right upper pulmonary lobe.
- Patchy areas of pleural calcifications are still seen, with minimal bilateral pleural thickening, more on the right side .
- No other patchy areas of consolidations or foci of pulmonary calcifications noted.
- No sizable mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusions.
- **Normal bones of the thoracic cage apart from spondylo-degenerative changes of the scanned spine and degeneration of L4-5 disc.**
- Average sized liver, measuring about 12.4 cm, showing normal parenchymal attenuation pattern.
- No sizable abnormally-enhancing hepatic focal lesions or dilated biliary radicles.
- Minimal subcapsular ,peri-hepatic fluid accumulation is seen.
- Patent PV.
- Average sized spleen,measuring about 8 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, both kidneys, both suprarenal glands, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- A small right ovarian cyst is seen and measures about 32x26mm in dimensions.
- The rest of the pelvic structures are unremarkable.

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-

-

IMPRESSIONS:

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient's Name:	**Magda Hussein Mohammed Talba**
Date:	**2026-08-04**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

A status follow up .compared with the lastly-done study at 08/08/2026. the current examination shows:

- A solid nodule in the right thyroid lobe.For US and US-guided FNAC.
- Bilateral apical pleural capping, more on the right side with fibrotic bands and ill-defined tiny pulmonary nodules still seen herein,with no appreciable changes seen,denoting a stable course.
- Mild subpleural reticulations in the anterior segment of right upper pulmonary lobe,likely to be a post-therapeutic sequelae.
- Patchy areas of pleural calcifications still seen, with minimal bilateral pleural thickening, more on the right side .
- Newly-developed minimal subcapsular ,peri-hepatic fluid accumulation.
- Mild right ovarian cyst.

Prof/Hanan El-Ahwal.

CT Scan
[[DOCTYPE:CT_SCAN]]